

# Acute GI Bleeding

Upper & lower — resuscitate, transfuse right, localize, scope

# Step 1 — Resuscitate & localize

Stabilize before you scope; the first Hb lags

| Clue                         | Points to                                |
|------------------------------|------------------------------------------|
| Hematemesis / coffee-ground  | Upper (above ligament of Treitz)         |
| Melena                       | Usually upper                            |
| BUN:Cr > 30 (no renal cause) | Upper source                             |
| Hematochezia + unstable      | Brisk upper bleed until proven otherwise |
| Hematochezia + stable        | Lower (diverticular, angioectasia)       |

**PEARL** A normal initial hemoglobin does not exclude a major bleed — resuscitate to vitals.

# Step 2 — Transfuse restrictively

Hb 7 target — Villanueva NEJM 2013

- Restrictive threshold Hb 7 g/dL (target 7–9) — better survival than liberal
- Higher threshold ~8 only with ACS or significant cardiovascular disease
- Variceal bleeding: restrictive is especially important
  - Do NOT over-transfuse plasma/platelets in varices — raises portal pressure
- Correct coagulopathy judiciously; don't delay endoscopy for a perfect INR

**PEARL** 'Topping up' to a normal hemoglobin increases rebleeding and death.

# Step 3 — Risk-stratify & prep the scope

Glasgow-Blatchford + pre-endoscopy meds

| Step                   | Detail                                              |
|------------------------|-----------------------------------------------------|
| Glasgow-Blatchford     | 0–1 = very low risk → consider outpatient endoscopy |
| IV PPI                 | Suspected ulcer/UGIB — start before endoscopy       |
| IV erythromycin 250 mg | ~20–90 min pre-scope — clears stomach, better view  |
| Endoscopy timing       | Within 24 h after resuscitation (not <12 h)         |

**PEARL** Resuscitate first — very early endoscopy in an unstable patient can worsen outcomes.

# The variceal pathway — treat it differently

- Suspect with cirrhosis / portal hypertension
- Start octreotide — lowers portal pressure
- Start prophylactic antibiotics (ceftriaxone) — reduces mortality
- Endoscopic band ligation is first-line definitive therapy
- Balloon tamponade or TIPS as rescue
- Keep transfusion restrictive (avoid raising portal pressure)

**PEARL** Antibiotics in variceal bleeding are a mortality intervention, not an afterthought.

# Lower GI bleed & what to avoid

## LGIB approach

- Most stop spontaneously
- Causes: diverticulosis, angioectasia, post-polypectomy, colitis, hemorrhoids
- Stable: colonoscopy after prep
- Ongoing brisk: CT angiography ± embolization
- Unstable: rule out upper source

## AVOID

- Liberal transfusion (target stays 7–9)
- 'Emergent' <12 h scope if under-resuscitated
- Withholding scope to chase a normal INR/platelets
- Anchoring on 'lower' when hematochezia + unstable

**PEARL** Unstable hematochezia → upper endoscopy first.

# Take-home points

- Resuscitate to vitals; the first hemoglobin lags
- Restrictive transfusion to Hb 7 (8 with ACS/CVD)
- Glasgow-Blatchford 0-1 → outpatient; IV PPI/erythromycin pre-scope; endoscopy ≤24 h
- Variceal: octreotide + antibiotics + band ligation
- Unstable hematochezia is an upper bleed until proven otherwise